3. Gestational Diabetes Mellitus (GDM)

This appears during pregnancy and disappears after pregnancy is terminated. Women with GDM have an increased future risk for progression to Type II or rarely Type I. The incidence of gestational diabetes is 15-17 percent in Indian pregnant women. In addition to ketoacidosis, pregnant woman with diabetes are more prone to preeclampsia, toxaemia and urinary tract infections. Uncontrolled diabetes during the first three months of pregnancy increases the risk of abortions and congenital malformations in the fetus. Elevated blood sugar should therefore be adequately controlled by dietary means and treatment with insulin.



Why the birth weight of the baby born to diabetic mothers are more?

- Mother's blood brings extra glucose to the fetus.
- Fetus make more insulin to handle the extra glucose.
- Extra glucose gets stored as fat and fetus becomes larger than normal.

4. Secondary Diabetes Mellitus

This may occur as a result of some other disorders such as Haemochromatosis, chronic pancreatitis, Down's syndrome, etc.

5. Malnutrition Related Diabetes Mellitus (MRDM)

Malnutrition-related diabetes mellitus (MRDM) is a rare type of diabetes associated with long term malnutrition. This type of diabetes is characterized by insulinopenia, insulin resistance, hyperglycemia and failure of the beta-cells in the pancreas. It is also known as tropical diabetes or tropical pancreatic diabetes mellitus. These patients are thin, young, severely hyperglycemic, but in contrast to IDDM do not have ketonuria and requires high doses of insulin for control.

6. Insulin Resistance

It is state in which a normal amount of insulin produces a subnormal amount of insulin response. These patients can present with diabetes or some of these patients can compensate for this defect with elevated serum levels and may have only impaired glucose tolerance. It can be due to:

- ❖ Genetic disorders
- ❖ Immune disorders
- ❖ Endocrine and metabolic conditions



Maturity Onset Diabetes of the Young (MODY) is a relatively rare type of Type 2 Diabetes that is characterized by an early age of onset (age 9-25) and autosomal dominant inheritance. Unlike type 2 Diabetes, often associated with insulin resistance, MODY is caused by a primary defect in pancreatic beta cell function resulting in decreased insulin secretion. Patients with MODY are not usually obese.

9.3. Causes

Primary or Idiopathic diabetes

Heredity: There is a familial tendency to develop Diabetes.

- ❖ If both parents are diabetic, all their children will be diabetic.
- ❖ If one parent is s diabetic, and the other is a diabetic carrier, half of their children will be potential diabetics.
- ❖ If one parent is diabetic and the other a non diabetic and also a non carrier, none of their children will be diabetic, but they all will be carriers.

However, environmental and other factors play a role in unmasking an underlying diabetes genotype and whether a person with a genetic predisposition actually develops the disease or not.

UNIT 9_new.indd 144 25-02-2019 15:37:14